DSM LAB · STUDY TOGETHER

The Unfolding Patient: role-play cards

Patients 1–12.

One person plays the patient; the other is the social worker doing an intake. **Print single-sided**, so the answer page never sits on the back of the patient card.

How to play

1. **Patient:** read the setting and your opening line aloud. Keep the rest of the card to yourself.
2. **Social worker:** interview. Ask about anything: onset and how long, sleep, substances and medications, medical history, safety, family, work or school, culture.
3. **Patient:** answer only what is asked, in your own words, using the "If asked about" table. Don't volunteer the items under "Only if asked".
4. **Social worker:** when ready (try 10 minutes), name a diagnosis and say what you'd do or check next.
5. Turn to the answer page together. Read the teaching point and the checkpoint question.

Each answer page follows its patient card. Keep it face down until the social worker commits to a diagnosis.

Scoring, if you want it

3 points for the right diagnosis, plus 1 for naming the rule-out (substance, medication or medical cause) before deciding, plus 1 for a safe, assess-first next step. Swap roles any time.

Playing it in the app

The same patients appear in DSM Lab → The Unfolding Patient, one clue at a time. If you play a card here first, the app version becomes a quick review.

These cases are original teaching material. Any resemblance to real people is coincidental.

Patient 1: The dean's-list student

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

Setting: Ana, 20, a community college student and part-time pharmacy clerk, at a community mental health intake. Her mother and older sister came with her.

> *"My mom made me come. I'm fine. I just need the people next door to stop talking about me."*

If asked about…

When did this start?	Around the end of summer, maybe ten weeks ago. Before that I was normal. Good grades, working at the pharmacy, hanging out with my cousins.
What do you hear?	Two voices through the wall, a man and a woman. They comment on what I'm doing, like 'she's brushing her teeth now.' Every day, worse at night.
What do you believe is going on?	They put a camera in the smoke detector. I took it apart. My mom says there's nothing in it, but I know what I heard.
Sleep	I'm up until three or four because the voices are loudest then. I'm tired, not wired. I don't feel full of energy.
Mood	I'm not sad, I'm scared. I've never had a time of feeling super high or powerful, and I've never been depressed for weeks. I still enjoy my K-dramas when it's quiet.
Alcohol, drugs, medications	I smoked weed in high school but stopped over a year ago because the pharmacy tests. I don't drink. No medications. The ER drug test three weeks ago was clean.
Medical history	The ER did blood tests and a scan of my head and said it was all normal. No seizures, no head injury, no fevers. I'm healthy.
Thoughts of suicide or harming others	No, I don't want to die and I've never hurt myself. The voices don't tell me to do anything. I would never hurt the neighbors. I just want them to stop.
Family history	My mom says an uncle in the Philippines had some kind of breakdown in his twenties. Nobody talks about it.
School and work	I stopped going to class about a month into the term, and I've called out of work for two weeks. My sister says I don't make sense when I talk. I start on one thing and end up somewhere else.
Culture and family context	My mom thought it was stress at first. She asked our priest to bless the house, and he told her to take me to a doctor. In my family we don't really talk about mental stuff.

Only if asked (don’t volunteer)

- Symptoms began only about ten weeks ago; before that she was fully functioning
- Stopped cannabis over a year ago; ER drug screen was negative
- Normal ER bloodwork and head scan
- An uncle had a 'breakdown' in his twenties

Patient 1: answer and debrief

DIAGNOSIS

Schizophreniform disorder

Voices plus disorganized speech meet the core psychosis requirement, which rules out delusional disorder. A clean screen and normal workup make a substance or medical cause unlikely, and there is no mood episode. Clue 6 flips schizophrenia to schizophreniform: the whole illness, with no earlier warning phase, has lasted about ten weeks, inside the 1 month to under 6 month window. Because she is still ill, it is recorded as provisional; if it reaches 6 months, the diagnosis becomes schizophrenia.

What had to be ruled out

- Substance/medication-induced psychotic disorder: cannabis stopped over a year ago, clean drug screen
- Psychotic disorder due to another medical condition: normal bloodwork and head scan
- Bipolar or depressive disorder with psychotic features: no manic or major depressive episode
- Brief psychotic disorder: symptoms have lasted more than 1 month
- Delusional disorder: hallucinations and disorganized speech are prominent
- Schizophrenia: total illness, including any warning phase, is under 6 months

Checkpoint: What would you ask or check NEXT?

A. Begin psychoeducation with Ana and her mother about schizophrenia and its treatment

B. Ask about drug and alcohol use and make sure she has had a medical evaluation, including a drug screen **(best)**

C. Refer her to a first-episode psychosis program for long-term coordinated specialty care

New psychosis in a young adult calls for ruling out substances and medical causes before naming a primary psychotic disorder. A is premature: no diagnosis is settled and the duration is unknown. C may be a good later step, but it skips the rule-outs that come first.

The case in six clues (as the app reveals it)

1. I stopped going to my classes about a month into the term. It's hard to be around people right now.
2. Mother: She says the neighbors put a camera in the smoke detector and talk about her through the wall. At night I hear her whispering back to them.
3. Sister: When she talks, her thoughts jump tracks. She starts a sentence about dinner and ends it on satellites, and I can't follow how she got there.
4. I don't use anything. I quit weed over a year ago when I started at the pharmacy, and the ER drug test three weeks ago was clean. They said my bloodwork and head scan were normal too.
5. I'm not sad and I'm not hyper. I just can't sleep because the voices get loud at night, and I still like my shows when it's quiet.
6. Mother: Last spring she made the dean's list and worked twenty hours a week. None of this started until about ten weeks ago. Before that she was completely herself.

What a skilled clinician weighs after each clue

1. Major depressive disorder, Social anxiety disorder, Schizophrenia
2. Schizophrenia, Substance/medication-induced psychotic disorder, Delusional disorder
3. Schizophrenia, Schizophreniform disorder, Substance/medication-induced psychotic disorder
4. Schizophrenia, Schizophreniform disorder, Bipolar I disorder
5. Schizophrenia, Schizophreniform disorder
6. Schizophreniform disorder

Sources: DSM-5-TR (APA, 2022), Schizophrenia Spectrum and Other Psychotic Disorders: Schizophreniform Disorder

Patient 2: The man with three labels

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

Setting: Marcus, 34, lives in supported housing and attends a church choir. He is at a new clinic for a second opinion; his case manager joins with his records.

> *"Another intake. Let me guess, you're going to give me a fourth label."*

If asked about…

Hospital history	First time at 26, then 29, then last year. Two places wrote schizophrenia, one wrote bipolar. Each time I was in about two weeks.
The high spells	When it hits I don't sleep for days, I talk a mile a minute, I give my money away. I believe God chose me as a prophet. The man's voice is usually there too. My last high ended in early April.
Voices when mood is steady	In April and May I felt level. Normal sleep, normal energy. But a man's voice kept talking about everything I did, every day, for about eight weeks. Then in June I crashed.
The low spells	In June I stayed in bed a month. No appetite, felt worthless, didn't shower. That's how my lows go, a month or more.
Overall course	Honestly, most of the last eight years I've been either up or down. The calm stretches don't last.
Sleep now	About seven hours. My mood is okay right now and the voice is quiet.
Alcohol, drugs, medications	I don't drink or use, never have. Every hospital drug test was clean. I'm on a mood stabilizer and an antipsychotic, and I take them most days.
Medical history	They checked my blood and thyroid every time. No head injuries, no seizures. I'm healthy.
Thoughts of suicide or harming others	During the low in June I thought everyone would be better off without me. I didn't plan anything and didn't do anything. Right now, no. I want my own apartment. The voice has never told me to hurt anybody.
Family history	My grandmother was in and out of the state hospital. Nobody ever said what for.
Work and daily life	I can't keep a job past a few months. When I'm level I sing in the choir and help set up for Sunday service.
Culture and context	At my church people talk about callings, but even they told me what I was saying went beyond that. My pastor drove me to the ER the first time. And I know how it goes: a big Black man talking loud, and they write schizophrenia.

Only if asked (don’t volunteer)

- The voice continued about 8 weeks in April and May while his mood was steady
- He has been in a mood episode for most of the 8 years
- Passive thoughts that others would be better off, during the June low; no plan or attempt

Patient 2: answer and debrief

DIAGNOSIS

Schizoaffective disorder

Psychosis during his highs looked like bipolar I with psychotic features. Clue 4 flips it: about eight weeks of voices with a steady mood is more than the 2 weeks of psychosis without a mood episode that schizoaffective disorder requires. Mood episodes covering most of the illness separate schizoaffective (bipolar type) from schizophrenia. Exam lesson: map psychosis against mood on a timeline, and check for bias, since Black clients are more often given a schizophrenia label while their mood symptoms are missed.

What had to be ruled out

- Substance/medication-induced psychotic or bipolar disorder: no use, clean screens at every admission
- Psychotic disorder due to another medical condition: normal bloodwork and thyroid
- Bipolar I disorder with psychotic features: psychosis lasted about 8 weeks with no mood episode
- Schizophrenia: mood episodes are present for most of the illness

Checkpoint: What would you ask or check NEXT?

A. Explain that this sounds like bipolar I disorder with psychotic features and begin psychoeducation

B. Suggest the prescriber add an antidepressant to help with the low spells

C. Ask whether the voices or the special beliefs ever continue when his mood is steady **(best)**

With substances and medical causes cleared, the deciding question is timing: does psychosis occur only during mood episodes, or also on its own? A settles on a diagnosis before that key fact is known. B is a treatment step, and an antidepressant can trigger a high in someone with manic episodes.

The case in six clues (as the app reveals it)

1. I've been in the hospital three times since I was 26. Every place gave me a different label, and I'm tired of it.
2. When it hits, I don't sleep for days. I'm giving money away, preaching on the bus, and I know God picked me to lead. A man's voice follows me around, telling me I'm chosen.
3. I don't drink or use anything, never really have. Every hospital ran a drug screen and bloodwork, thyroid too, and all of it came back clean.
4. Case manager: His last high spell ended in early April. Through April and May his mood was steady and he slept fine, but he kept hearing a man's voice commenting on everything he did.
5. That voice was there every day for about eight weeks, not just a night or two. Then in June I crashed and stayed in bed for a month.
6. Case manager: Looking over eight years of records, he has been in a high or a low spell for most of that time. The steady stretches are short.

What a skilled clinician weighs after each clue

1. Bipolar I disorder, Schizophrenia, Schizoaffective disorder, Major depressive disorder
2. Bipolar I disorder, Schizoaffective disorder, Substance/medication-induced bipolar disorder
3. Bipolar I disorder, Schizoaffective disorder
4. Schizoaffective disorder, Bipolar I disorder, Schizophrenia
5. Schizoaffective disorder, Schizophrenia
6. Schizoaffective disorder

Sources: DSM-5-TR (APA, 2022), Schizophrenia Spectrum and Other Psychotic Disorders: Schizoaffective Disorder

Patient 3: The tired accountant

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

Setting: Jiyeon, 41, a corporate accountant and mother of two, referred by her primary care doctor before any antidepressant is started.

> *"My doctor said to talk to someone before she writes me a prescription. I just want to feel like myself again."*

If asked about…

When did this start and how long?	About five weeks ago. It's every day, most of the day.
Symptoms	I sleep ten hours and wake up tired. I'm eating more, mostly sweets. I feel useless and slow, and I reread the same spreadsheet three times.
Past high-energy periods	Twice, maybe two and four years ago, I had about six days where I slept three hours and felt great. I repainted the whole house in a week and talked my husband's ear off. I thought that was just the real me finally showing up.
Did the high spells cause problems?	Not really. I went to work and got a lot done. I didn't spend wildly or get in trouble, nobody had to help me, and I never heard or saw things that weren't there.
Earlier low spells	At 26, after grad school, I had a low spell like this for a couple of months. It lifted on its own.
Sleep	Ten hours right now, and I still nap on weekends.
Alcohol, drugs, medications	A glass of wine on holidays. No drugs. No medications, not even birth control, and I have never taken an antidepressant. I wasn't on anything during the high spells either.
Medical history	My thyroid and blood count were normal last week. No major health problems.
Thoughts of suicide	Sometimes I think my family would manage fine without me. But I don't want to die, and I would never do that to my kids. No plan, nothing like that.
Family history	My father had big moods. Huge projects, then weeks in his room. Nobody ever called it anything.
Work	I'm behind at work for the first time in my career, and my boss noticed.
Culture and context	In my Korean family, you push through. My mother says I should pray more and eat better. Coming here feels like failing.

Only if asked (don’t volunteer)

- Two past spells of about six days of high energy and little sleep
- A depressive episode at age 26
- Father with possible mood episodes
- Passive thoughts that her family would manage without her; no plan or intent

Patient 3: answer and debrief

DIAGNOSIS

Bipolar II disorder

The current episode meets major depression, and the medical screen is normal. Clue 4 flips the picture: about six days of little sleep, high energy and nonstop talk, noticed by others, without marked impairment, hospitalization or psychosis, is a hypomanic episode. Hypomania plus a major depressive episode and no lifetime mania is bipolar II. Exam lesson: always ask about past highs before depression treatment, and remember hypomania needs at least 4 days.

What had to be ruled out

- Depressive disorder due to another medical condition: normal thyroid and blood count
- Substance/medication-induced bipolar disorder: no medications or drugs during the high spells
- Major depressive disorder: past hypomanic episodes rule it out
- Bipolar I disorder: no episode with marked impairment, hospitalization or psychosis
- Cyclothymic disorder: full depressive and hypomanic episodes have occurred

Checkpoint: What would you ask or check NEXT?

A. Ask whether she has ever had a stretch of days with unusually high energy and little need for sleep **(best)**

B. Tell her doctor the picture fits depression so an antidepressant can be started

C. Begin weekly CBT for depression

Before depression is treated, screen for past hypomania or mania. Depression is often the first bipolar episode people report, and an antidepressant alone can trigger a switch. B skips that screen. C is a reasonable treatment, but it starts before the diagnosis is clear.

The case in six clues (as the app reveals it)

1. I've felt heavy and slow for about five weeks. I sleep ten hours and still drag myself through the day.
2. I've lost interest in everything, even my kids' soccer games. I feel useless at work, I can't focus, and I cry in the car. It's like this every day.
3. My doctor checked my thyroid and blood count last week, and both were normal. I have a glass of wine on holidays and I'm on no medications.
4. Husband: Two springs ago she had a stretch where she slept three hours a night, repainted the whole house, and talked nonstop. She was fun, honestly. Her office called her a whirlwind.
5. That spell lasted about six days. I didn't miss work or get in any trouble, nobody had to take care of me, and I never saw or heard strange things.
6. It's happened twice like that, and I had another long low spell in my twenties. In between, I'm my normal self for long stretches.

What a skilled clinician weighs after each clue

1. Major depressive disorder, Depressive disorder due to another medical condition, Adjustment disorder
2. Major depressive disorder, Depressive disorder due to another medical condition
3. Major depressive disorder, Bipolar II disorder, Bipolar I disorder
4. Bipolar II disorder, Bipolar I disorder, Cyclothymic disorder
5. Bipolar II disorder, Cyclothymic disorder
6. Bipolar II disorder

Sources: DSM-5-TR (APA, 2022), Bipolar and Related Disorders: Bipolar II Disorder

Patient 4: The retired bus driver

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

Setting: Roberto, 68, a retired city bus driver and widower, brought to a senior outpatient clinic by his daughter.

> *"I don't know why my daughter dragged me here. I have meetings to take. Business is booming!"*

If asked about…

When did this start?	Couple of weeks? I woke up one morning with all these ideas, and they haven't stopped.
Sleep	Two, three hours a night. I don't need more. I feel fantastic.
Spending and plans	I bought a food truck and put deposits on two more. I drove to the city with my savings to buy a fleet. The police were very rude about it.
Medications	An inhaler for my lungs and a blood pressure pill I've taken for years. The steroid pills last month, five days, done long before this started. Nothing new since.
Alcohol and drugs	A beer at a barbecue now and then. No drugs ever. I haven't had a drink in weeks; no time.
Medical history	COPD and blood pressure. The doctor did blood work, thyroid, a scan of my head. All good. I'm sharp as a tack. Ask me anything.
Past episodes	At 31 I had a spell like this. I quit my job to start a band and ended up in the hospital for ten days. They gave me lithium. I stopped it after a year and felt fine for decades.
Thoughts of suicide or harming others	Kill myself? I'm just getting started! No. And I'm not going to hurt anybody. I just want people to keep up with me.
Family history	My mother had 'nervios.' Some years she stayed in bed, some years she sewed all night and didn't sleep. In Michoacán nobody took her to a doctor for that.
Daily functioning	My daughter took my car keys yesterday, which is ridiculous. I haven't paid my bills this month; I've been too busy.
Culture and context	In my family you don't air your business. My wife made me promise never to tell the kids about the hospital. It's my money and my life.

Only if asked (don’t volunteer)

- Hospitalized at 31 for a similar episode; took lithium about a year
- Steroid pack ended about a month before symptoms began
- Mother's pattern of long low spells and sleepless high spells

Patient 4: answer and debrief

DIAGNOSIS

Bipolar I disorder

Late-life mania with recent steroid exposure points first to a medication or medical cause, and clue 3 makes that the lead. The workup flips it back: the steroid ended a month before onset, labs and scan are normal, and he is alert and oriented, so neither delirium nor a medication effect explains it. A week-plus of elevated mood with marked impairment is a manic episode, and the hospitalized episode at 31 shows long-standing bipolar I. Exam lesson: rule out medical and substance causes first, then let the history decide.

What had to be ruled out

- Substance/medication-induced bipolar disorder: steroid course ended about a month before onset
- Delirium: alert, oriented and attentive; normal labs
- Bipolar and related disorder due to another medical condition: normal thyroid, labs and brain scan
- Major neurocognitive disorder: acute change, no gradual cognitive decline
- Bipolar II disorder: current and past episodes involve marked impairment and hospitalization

Checkpoint: What would you ask or check NEXT?

A. Refer him to a psychiatrist to start a mood stabilizer

B. Arrange an urgent medical evaluation that includes a full review of his medications **(best)**

C. Help his daughter start a conservatorship to protect his savings

An apparent first manic picture after age 40, with a recent steroid course, calls for a medical and medication workup first. A treats a primary bipolar disorder that has not been established. C may matter later, but it comes after assessment and is far more restrictive.

The case in six clues (as the app reveals it)

1. Daughter: Dad's been different for about two weeks. He's up at four in the morning rearranging the garage and calling people at all hours.
2. I've never felt better in my life. I bought a food truck on Tuesday, and I'll have five by summer. Sleep is for people with nothing to do.
3. Daughter: He's 68 and has never seen a psychiatrist that I know of. Last month his lung doctor gave him steroid pills for a COPD flare.
4. Daughter: The lung doctor says it was a five-day pack that ended a full month before this began. His labs, thyroid and brain scan are normal, and he knows the date and keeps up with every word we say.
5. Daughter: Two nights ago he drove three hours with his savings to 'buy a fleet,' and police found him at a casino at 3 a.m. He's been like this every day.
6. When I was 31 I had a spell just like this. I quit my job to start a band, didn't sleep for a week, and ended up in the hospital for ten days. My late wife never told the kids.

What a skilled clinician weighs after each clue

1. Bipolar I disorder, Delirium, Substance/medication-induced bipolar disorder, Major neurocognitive disorder
2. Bipolar I disorder, Substance/medication-induced bipolar disorder, Delirium
3. Substance/medication-induced bipolar disorder, Bipolar I disorder, Delirium
4. Bipolar I disorder, Substance/medication-induced bipolar disorder
5. Bipolar I disorder, Substance/medication-induced bipolar disorder
6. Bipolar I disorder

Sources: DSM-5-TR (APA, 2022), Bipolar and Related Disorders: Bipolar I Disorder; DSM-5-TR (APA, 2022), Bipolar and Related Disorders: Substance/Medication-Induced Bipolar and Related Disorder

Patient 5: The lab student

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

Setting: Minh, 24, a chemistry graduate student, at the university counseling center. His mother joins by phone for part of the visit.

> *"My advisor said I should come. I don't really know what to say. I just feel flat."*

If asked about…

When did this start?	About a month ago, maybe a bit more. It came on over a week or two. I've never felt like this before.
Mood and interest	Low most of the day. Nothing seems worth doing. I used to love running and I haven't gone in weeks.
Sleep and appetite	I wake up at four and can't get back to sleep. I'm not hungry, and I've lost about eight pounds.
Medications	Just an acne pill, isotretinoin. I started it about ten weeks ago. The dermatologist stopped it two weeks ago after my mom called her. Nobody asked me about mood when I started it.
Alcohol and drugs	I don't drink or smoke. No drugs. Too much coffee, maybe two cups.
Medical history	Healthy. My checkup in September was normal, thyroid too. No head injuries.
Thoughts of suicide	Some nights I've thought it would be easier not to wake up. I've never had a plan or done anything, and I wouldn't. I don't want to do that to my parents. It's been less since I stopped the pill.
Since stopping the pill	I'm sleeping a little better. I ate a whole bowl of pho on Sunday. I'm not back to normal, but it's lifting.
Past mood history	Never had a low spell or a time of being wired and not sleeping. Even finals never did this to me.
Family history	Nobody in my family has had depression that we know of. My grandfather drank, I think.
School functioning	I've missed lab meetings and my experiment is behind. My advisor noticed I wasn't myself.
Culture and context	My parents came from Vietnam. We don't really say 'depressed.' My mom thinks I'm worn out from studying too much, but she was the one who called the dermatologist.

Only if asked (don’t volunteer)

- Started isotretinoin about 10 weeks ago; mood dropped a few weeks later
- Passive thoughts of not waking up, with no plan or intent
- Mood has been lifting since the medication was stopped 2 weeks ago

Patient 5: answer and debrief

DIAGNOSIS

Substance/medication-induced depressive disorder

The symptoms meet a major depressive episode, so MDD leads at first. Clue 4 flips it: the low mood began weeks after starting isotretinoin, which carries warnings about depression and suicidal thoughts, in someone with no prior episodes. Improvement within two weeks of stopping supports the medication as the cause; if symptoms lasted well over a month after stopping, an independent depression would be more likely. Exam lesson: substances and medications are the first rule-out; when a drug is suspected, the client's prescriber needs to know right away.

What had to be ruled out

- Major depressive disorder: onset tied to the medication, no prior episodes, improving after it stopped
- Depressive disorder due to another medical condition: normal checkup and thyroid
- Adjustment disorder: no identifiable stressor
- Persistent depressive disorder: about one month, not 2 years
- Bipolar disorders: no history of high spells

Checkpoint: What would you ask or check NEXT?

A. Ask about any new prescriptions or other medications started in the past few months **(best)**

B. Refer him for an antidepressant evaluation

C. Start behavioral activation and schedule weekly sessions

A first depressive episode with no history and no stressor calls for checking substances and medications before calling it a primary depression. B and C treat a primary depression that has not been established, and B could add a drug before the likely cause is found.

The case in six clues (as the app reveals it)

1. For about a month I've had no energy, and nothing seems worth doing.
2. I'm not sleeping well, I've lost about eight pounds without trying, and I keep thinking I'm a disappointment. My focus in the lab is shot.
3. I've never been like this before, not even in my hardest semesters. I don't drink or smoke, and nothing bad happened this fall.
4. The only new thing is an acne pill from my dermatologist. I started it about ten weeks ago, and the low mood began a few weeks after that.
5. Mother: He has always been a steady, easygoing kid. No low spells, no wild highs, and his checkup with the thyroid test was normal in September.
6. Mother: When we told the dermatologist, she stopped the pill. That was two weeks ago. He's not himself yet, but he's eating again and joking a little.

What a skilled clinician weighs after each clue

1. Major depressive disorder, Adjustment disorder, Persistent depressive disorder
2. Major depressive disorder, Substance/medication-induced depressive disorder, Depressive disorder due to another medical condition
3. Major depressive disorder, Substance/medication-induced depressive disorder, Depressive disorder due to another medical condition
4. Substance/medication-induced depressive disorder, Major depressive disorder
5. Substance/medication-induced depressive disorder, Major depressive disorder
6. Substance/medication-induced depressive disorder

Sources: DSM-5-TR (APA, 2022), Depressive Disorders: Substance/Medication-Induced Depressive Disorder

Patient 6: The coffee cup on Sundays

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

Setting: Eleni, 63, a retired seamstress, at a community counseling center. Her daughter-in-law made the appointment.

> *"My daughter-in-law found you. I don't know what talking will do. Nothing brings him back."*

If asked about…

When did your son die?	Fourteen months ago. A truck ran a red light. He was 38.
What a day is like	I think about him all day, every day. I play his voicemail at night. I set his coffee cup out on Sundays like he's coming to dinner.
Sense of self	I was his mother. Now who am I? Part of me went into the ground with him. It still doesn't feel real.
Sleep and appetite	I sleep six or seven hours with the TV on. I eat because my sister brings food. My weight is the same.
Guilt or worthlessness	No. I don't think I'm bad or worthless. I was a good mother to him. I'm not guilty. I just ache for him.
Memories of the crash	I wasn't there. I don't have nightmares about it and I don't jump at noises. I go to his grave every week. I want to be near him, not away from reminders.
Alcohol, drugs, medications	A blood pressure pill. A small glass of wine at Easter. My doctor offered sleeping pills and I said no.
Medical history	Blood pressure and bad knees. My checkup in the summer was fine, thyroid normal.
Thoughts of suicide	Sometimes I wish God would take me so I could be with him. But I would never do anything to myself. It's a sin, and I have my granddaughter. No plan, never.
Family history	My mother wore black for years after my father died, but she still cooked for the whole parish. Nobody in my family had depression that I know of.
Daily life	I stopped volunteering at the church kitchen and I don't go to coffee with my friends anymore. Everything feels pointless without him. I'm so lonely it hurts.
Culture and faith	We are Greek Orthodox. We had the forty-day and one-year memorials, and they helped a little. But Father Petros says, gently, that I'm stuck. My sisters say the same.

Only if asked (don’t volunteer)

- The death was 14 months ago
- Self-worth intact; no guilt; sleep and appetite steady
- Stopped volunteering and seeing friends
- Passive wish to be with her son, with no plan or intent

Patient 6: answer and debrief

DIAGNOSIS

Prolonged grief disorder

Intense yearning, disbelief and a lost sense of self can be normal early in grief. Clue 4 flips it: the death was 14 months ago, past the 12-month adult threshold, and the reaction exceeds what her own faith community expects. Her self-worth is intact and sleep and appetite are steady, so major depression is not the better fit. Exam lesson: prolonged grief disorder is new in DSM-5-TR and needs at least 12 months since the death in adults (6 months in children and teens).

What had to be ruled out

- No mental disorder (normal or expected reaction): over 12 months, beyond cultural norms, impairing
- Major depressive disorder: self-worth intact, sleep and appetite steady, pain centered on her son
- Posttraumatic stress disorder: no intrusive memories of the crash, no avoidance or hyperarousal
- Adjustment disorder: not used for bereavement that meets prolonged grief disorder

Checkpoint: What would you ask or check NEXT?

A. Reassure her that grief has no timetable and suggest a bereavement support group

B. Refer her for an antidepressant evaluation

C. Ask exactly how long ago her son died **(best)**

Yearning, disbelief and a shaken identity fit both normal grief and prolonged grief disorder; the time since the death decides between them (at least 12 months for adults). A assumes normal grief before that fact is known. B treats depression, which has not been established.

The case in six clues (as the app reveals it)

1. I can't seem to get back to my life. Most days I don't leave the house except for church.
2. My son Nikos died in a car crash. I play his voicemail every night, and I still set out his coffee cup on Sundays.
3. It still doesn't feel real. Part of me went into the ground with him, and I don't know who I am now if I'm not his mother.
4. It was fourteen months ago. We did the forty-day and one-year memorials, and my priest and my sisters say I should be coming back to the living by now.
5. I eat because my sister brings food, and I sleep fine with the TV on. I don't think I'm a bad person or worthless. I just miss him, every single day.
6. I quit volunteering at the church kitchen and I don't see my friends. Nothing means anything without him, and I'm so lonely it hurts.

What a skilled clinician weighs after each clue

1. Major depressive disorder, Adjustment disorder, Agoraphobia
2. No mental disorder (normal or expected reaction), Major depressive disorder, Posttraumatic stress disorder
3. Prolonged grief disorder, No mental disorder (normal or expected reaction), Major depressive disorder
4. Prolonged grief disorder, Major depressive disorder
5. Prolonged grief disorder, Major depressive disorder
6. Prolonged grief disorder

Sources: DSM-5-TR (APA, 2022), Trauma- and Stressor-Related Disorders: Prolonged Grief Disorder; Prigerson et al. (2021), World Psychiatry 20(1), Validation of the DSM-5-TR criteria for prolonged grief disorder

Patient 7: The children on the couch

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

Setting: Hiroshi, 76, a retired high school chemistry teacher, at a geriatric assessment clinic with his wife of 50 years.

> *"My wife thinks I'm losing my marbles. Some days I think she might be right."*

If asked about…

When did this start?	A little over a year ago. Good days and bad days. On bad days I feel like I'm in a fog and lose my train of thought.
The visions	Small children, two or three, on the couch most evenings. Sometimes a cat we don't own. They don't talk or tell me to do anything. They don't scare me anymore.
Sleep	Wife: For maybe eight years he's yelled and swung his arms in his sleep. Now he's very sleepy in the day and naps twice.
Movement	My hands tremble and I shuffle. I fell once in the garden. That started a few months ago, after the memory trouble and before that shot in the ER.
Medications, alcohol, drugs	A blood pressure pill and a baby aspirin. No sleeping pills. A beer on Sundays. Wife: The ER shot made him stiff and out of it for two days. I was terrified.
Medical history	Wife: His doctor checked urine, blood, B12 and thyroid. All normal. No strokes, no fevers, no falls on his head.
Daily functioning	Wife: I've handled the checkbook and his pills since spring, after he paid the electric bill twice. He still dresses and showers himself. He stopped driving in the summer.
Mood	I'm frustrated, not depressed. I still enjoy my bonsai and the grandkids.
Thoughts of suicide or harming others	No, I don't want to die. I'd never hurt anyone. Wife: He's never been rough when he's awake. The kicking in his sleep is the only thing.
Family history	Nobody in my family had memory problems that I know of. My parents died fairly young.
Culture and context	My parents were in the camps during the war. We were raised with 'gaman,' to endure and not complain. So I didn't tell anyone about the children on the couch for months.

Only if asked (don’t volunteer)

- Years of acting out dreams in sleep
- Severe stiffness and sedation after one antipsychotic dose
- Tremor and shuffling began after the memory problems
- Wife took over bills and medications

Patient 7: answer and debrief

DIAGNOSIS

Neurocognitive disorder with Lewy bodies

Fluctuating attention and visual hallucinations first suggest delirium. Clue 4 flips it: the workup is normal and the course is over a year, not hours to days. Fluctuating cognition, well-formed visual hallucinations, years of acting out dreams, parkinsonism that came after the cognitive decline, and a severe reaction to one antipsychotic dose point to Lewy bodies. He needs help with bills and medications, so it is a major NCD. Exam lesson: new visual hallucinations in an older adult point to a medical or neurocognitive cause, not schizophrenia; rule out delirium, then think Lewy bodies and use great caution with antipsychotics.

What had to be ruled out

- Delirium: normal urine, blood and medication review; slow course over more than a year
- Major neurocognitive disorder of the Alzheimer type: early fluctuations, visual hallucinations and dream enactment fit Lewy bodies better
- Major neurocognitive disorder due to Parkinson's disease: tremor and shuffling began after the cognitive decline, not years before it
- Psychotic disorder due to another medical condition or a primary psychotic disorder: hallucinations are part of the cognitive syndrome
- Major depressive disorder: mood is frustrated, not depressed; interest preserved

Checkpoint: What would you ask or check NEXT?

A. Arrange a medical evaluation to look for infection, medication effects or other causes of delirium **(best)**

B. Ask his doctor about an antipsychotic to stop the visions

C. Help his wife begin planning a move to memory care

Fluctuating attention and new visual hallucinations in an older adult must be checked for delirium first, because delirium is a medical emergency. B is risky: people with this pattern can react severely to antipsychotics. C plans for a long-term condition before the cause is known.

The case in six clues (as the app reveals it)

1. Wife: Some days he's as sharp as ever, and other days he stares off and can't follow a conversation.
2. In the evenings I see small children sitting on the couch. They never speak. They look completely real, though I know they can't be there.
3. Wife: For years before any of this, he shouted and swung his arms in his sleep, like he was acting out his dreams. Last month he kicked me out of bed.
4. Wife: His doctor checked his urine, blood, B12 and thyroid, and nothing is new in his pills. This has crept along for over a year, and I've had to take over the bills and his pills.
5. My hands shake now and I shuffle when I walk. That started a few months ago, well after the memory trouble.
6. Wife: In the ER last month they gave him one shot of an antipsychotic for the visions. He went stiff as a board and could barely wake up for two days.

What a skilled clinician weighs after each clue

1. Delirium, Major neurocognitive disorder, Major depressive disorder, Neurocognitive disorder with Lewy bodies
2. Delirium, Neurocognitive disorder with Lewy bodies, Psychotic disorder due to another medical condition
3. Neurocognitive disorder with Lewy bodies, Delirium, Major neurocognitive disorder
4. Neurocognitive disorder with Lewy bodies, Major neurocognitive disorder
5. Neurocognitive disorder with Lewy bodies, Major neurocognitive disorder
6. Neurocognitive disorder with Lewy bodies

Sources: DSM-5-TR (APA, 2022), Neurocognitive Disorders: Major or Mild Neurocognitive Disorder With Lewy Bodies; McKeith et al. (2017), Neurology 89(1), Fourth consensus report of the DLB Consortium

Patient 8: The man behind the curtain

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

Setting: Layla, 49, office manager, on an orthopedic unit after a car crash broke her leg. The nurse asks for a consult about new agitation. The team wants to know what is driving tonight's change.

> *"Please, I need to go home. Something is wrong in this room, and nobody listens to me."*

If asked about…

When did this start?	Last night. I can't stop shaking, I'm sweating, and my heart is racing. I haven't slept since the crash.
What are you seeing?	There's a man behind that curtain. He doesn't talk. The nurse says no one is there, and part of me knows she's right, but I keep seeing him. Last night the floor looked like it was moving.
Where are you and what day is it?	I'm in the hospital. I broke my leg in the crash on Thursday. Today is Saturday.
Medications	Alprazolam, three times a day, for six years. My family doctor prescribes it. I haven't had any since the crash. The ER only got the list from my heart doctor, and I didn't say anything.
How you take it	Exactly as written. I never take extra and never run out early. My doctor started it after my father died; I couldn't sleep or stop crying. I just kept taking it.
Alcohol and drugs	I don't drink at all, never have. It was never part of my life. No other drugs.
Medical history	High blood pressure. I've never had a seizure. They said my head scan was fine.
Mood and worry before the crash	I was fine. Busy, but fine. I don't lie awake worrying about things.
Thoughts of suicide or harming others	No. I want to see my kids. I'm not going to hurt anyone. I'm scared, that's all.
Family history	My mother worried a lot. Nobody in my family drank or used drugs.
Work and daily life	I run the office at my brother's dental practice. I drive my kids everywhere. I was doing everything until Thursday.
Culture and context	My husband and sisters are here all the time, and my sister prays with me. In my family a 'nerve pill' is something to be ashamed of. That's why I didn't tell the ER.

Only if asked (don’t volunteer)

- Takes alprazolam three times daily for six years, exactly as prescribed
- Last dose was the morning of the crash
- Did not tell the ER about the pill out of embarrassment

Patient 8: answer and debrief

DIAGNOSIS

Sedative, hypnotic or anxiolytic withdrawal

Autonomic signs with a hallucination and a clear mind early in an admission suggest alcohol withdrawal. Clue 4 flips it: she stopped a daily benzodiazepine abruptly at admission and does not drink. Because she took it exactly as prescribed, tolerance and withdrawal do not count toward a use disorder. Exam lesson: sedative and alcohol withdrawal can cause seizures and death, so medication histories must include prescribed sedatives, and the medical team must act fast. Treatment is medical: the team restarts or substitutes the medication, and any later reduction is a slow, supervised taper, never an abrupt stop.

What had to be ruled out

- Alcohol withdrawal: does not drink, confirmed by her husband
- Sedative, hypnotic or anxiolytic use disorder: taken exactly as prescribed; tolerance and withdrawal alone do not count
- Delirium: alert, oriented and attentive; normal labs, oxygen and scan (watch closely, since withdrawal can progress)
- Acute stress disorder and panic disorder: do not explain the vital signs or the timing after stopping the medication

Checkpoint: What would you ask or check NEXT?

A. Screen her for acute stress disorder related to the crash

B. Suggest a sitter and an antipsychotic to manage her agitation

C. Ask what she drinks or takes at home, including prescribed pills, and alert the medical team now **(best)**

Tremor, a fast pulse, sleeplessness and a visual hallucination two days into a hospital stay suggest withdrawal, which can cause seizures. Finding the substance and telling the medical team comes first. A explains none of the physical signs. B treats the behavior without finding the cause.

The case in six clues (as the app reveals it)

1. Nurse: Since last night she's been shaky, sweaty and awake all night. She keeps saying she has to get out of here.
2. There's a man standing behind that curtain. He's been there for hours. The nurse says nobody is there, so why do I keep seeing him?
3. Nurse: Pulse 114, blood pressure up, hands trembling. But she knows she's in the hospital, knows the date, and can repeat back what we tell her.
4. I haven't had my alprazolam since the crash. I've taken it three times a day for six years, from my family doctor. It wasn't on the list the ER got.
5. Husband: She doesn't drink at all, never has. She never takes extra or runs out early; the pharmacy can show she refills right on time.
6. Nurse: Her last dose was the morning of the crash, about 60 hours ago. Her labs, oxygen and head scan are normal, and she is still oriented.

What a skilled clinician weighs after each clue

1. Panic disorder, Delirium, Alcohol withdrawal, Generalized anxiety disorder
2. Delirium, Alcohol withdrawal, Sedative, hypnotic or anxiolytic withdrawal
3. Alcohol withdrawal, Sedative, hypnotic or anxiolytic withdrawal, Delirium
4. Sedative, hypnotic or anxiolytic withdrawal, Sedative, hypnotic or anxiolytic use disorder, Alcohol withdrawal
5. Sedative, hypnotic or anxiolytic withdrawal, Delirium
6. Sedative, hypnotic or anxiolytic withdrawal

Sources: DSM-5-TR (APA, 2022), Substance-Related and Addictive Disorders: Sedative, Hypnotic, or Anxiolytic Withdrawal; SAMHSA TIP 45, Detoxification and Substance Abuse Treatment: sedative-hypnotic withdrawal

Patient 9: The unanswered text

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

Setting: Sean, 29, an auto mechanic, at an outpatient clinic. His older brother told him to come. His mother joins for part of the second visit.

> *"My brother said if I didn't see somebody he was done with me. So. Here I am."*

If asked about…

How long do the mood shifts last?	Hours, not days. Fine at breakfast, wrecked by lunch if someone blows me off. By the next day I'm usually okay.
Sleep	Seven or eight hours, always. I've never had nights of no sleep where I felt great.
Spending and impulsive behavior	I bought the motorcycle the day after my ex left. I drive way too fast when I'm upset. Sometimes I eat until I feel sick.
Self-harm	When things blow up I hurt myself on purpose, maybe twice a month. Last week was the last time. I take care of it after. I'm not trying to die when I do it.
Suicide history and current thoughts	At 19, after a breakup, I made an attempt and spent a night in the ER. Nothing since. Some nights when I'm alone and wrecked I think about dying, but it passes by morning. Right now I don't want to die, I have no plan, and my dog and my nephew are reasons I stay.
Relationships	Every relationship starts as the best thing ever and ends in a screaming fight. I beg people not to leave, then I'm the one who blows it up. I punch walls. I've never hit a person.
Sense of self	I don't really know who I am. I change jobs, friends, how I dress. Inside it feels empty most of the time.
Alcohol, drugs, medications	I drink on weekends, sometimes too much after a fight. No drugs. I'm not on any medications.
Medical history	Healthy. My physical last year was fine. No head injuries, no seizures.
Family history	My dad drank and left when I was seven. My mom raised my brother and me.
Work	Five jobs in three years. I'm a good mechanic. I just quit when a boss gets on me.
Culture and context	In my family we don't talk about feelings; we joke. Where I'm from, guys don't go to therapy.

Only if asked (don’t volunteer)

- Suicide attempt at 19
- Self-harm about twice a month, most recently last week
- Mood shifts last hours and follow interpersonal triggers
- Chronic feelings of emptiness

Patient 9: answer and debrief

DIAGNOSIS

Borderline personality disorder

Big highs, lows and impulsive spending first suggest bipolar II or cyclothymia. Clue 3 flips it: his shifts last hours, follow relationship triggers, and never come with reduced need for sleep. Self-harm, frantic efforts to avoid abandonment, rage, emptiness and a pattern since early adolescence point to borderline personality disorder; rage and emptiness, not appeasement, separate it from dependent. Exam lesson: mood shifts lasting hours and reacting to people suggest BPD; bipolar episodes last days to weeks. Always assess safety, and confirm a personality pattern with history and collateral, not from one session.

What had to be ruled out

- Bipolar II disorder: no episodes of 4 or more days with reduced need for sleep
- Cyclothymic disorder: his highs are brief, trigger-driven and lack reduced need for sleep; shifts last hours, not days
- Major depressive disorder: lows last about a day and follow triggers
- Dependent personality disorder: reacts to abandonment with rage and self-harm, not appeasement

Checkpoint: What would you ask or check NEXT?

A. Ask about thoughts of suicide and self-harm, now and in the past **(best)**

B. Tell him the pattern fits borderline personality disorder and refer him to DBT

C. Ask the prescriber about a mood stabilizer for the mood swings

Intense, fast-shifting moods after interpersonal triggers raise the risk of self-harm and suicide, and safety comes before diagnosis or treatment. B labels a personality disorder after one session without a full history or safety check. C treats a mood disorder that the timing argues against.

The case in six clues (as the app reveals it)

1. My moods are all over the place. My last girlfriend said being with me was exhausting.
2. Some mornings I'm on top of the world, and once I put a $3,000 motorcycle on a credit card I can't pay. Other times I can't get out of bed.
3. The highs and lows don't last days. It flips in an afternoon. If a text goes unanswered I go from fine to wrecked in ten minutes, and by the next morning I'm okay.
4. When things blow up, I hurt myself on purpose. It's not about dying; it makes the noise in my head stop. The last time was last week after a fight with my brother.
5. I need my eight hours even on good days, and I've never gone days without sleep feeling great. What I can't stand is people pulling away. I once called a girlfriend twenty times in a night begging her not to leave.
6. Mother: It's been like this since ninth grade. Friendships that start intense and end in screaming, holes punched in walls, five jobs in three years. He tells me he feels empty inside, like nobody's home.

What a skilled clinician weighs after each clue

1. Bipolar II disorder, Cyclothymic disorder, Borderline personality disorder, Major depressive disorder
2. Bipolar II disorder, Cyclothymic disorder, Borderline personality disorder
3. Borderline personality disorder, Cyclothymic disorder, Bipolar II disorder
4. Borderline personality disorder, Bipolar II disorder
5. Borderline personality disorder, Dependent personality disorder
6. Borderline personality disorder

Sources: DSM-5-TR (APA, 2022), Personality Disorders: Borderline Personality Disorder; Stanley & Brown (2012), Cognitive and Behavioral Practice 19, Safety Planning Intervention

Patient 10: The new team lead

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

Setting: Priya, 31, a software engineer recently promoted to team lead, at an employee assistance program visit.

> *"My manager suggested the EAP. I keep having these attacks at work, and I think it's going to cost me the promotion."*

If asked about…

What happens during an attack?	My heart pounds, I sweat, my hands shake, my face gets hot, and I feel like I'll pass out. It peaks in a few minutes and fades in maybe twenty.
When and where do they happen?	Only at work, in meetings when I have to present, or at team lunches when people turn to me. Never at home, never out of the blue, never driving or in a crowded store.
What are you afraid will happen?	That they'll see me shake or blush and think I'm a fraud who doesn't deserve the job.
How long has this been going on?	The attacks started in the spring when I got promoted. But the fear of being watched goes back to college. I dropped a class to skip a speech.
Avoidance	I eat at my desk. I schedule my one-on-ones by chat when I can. I skipped the holiday party last year.
Do you worry about having another attack at other times?	Not really. I'm not afraid of going places alone or of attacks at home. I only dread the meetings.
Sleep	Fine, except the night before a presentation.
Caffeine, alcohol, drugs, medications	One coffee in the morning. Sometimes a glass of wine before a work dinner to calm down. No drugs, no medications, no supplements.
Medical history	My doctor did an EKG and checked my thyroid last month. Both normal. Healthy otherwise.
Thoughts of suicide	No, never. I'm frustrated, not hopeless.
Family history	My mother is very shy. She never spoke at parent meetings.
Culture and context	My parents are from Chennai. They're so proud of the promotion, and they don't know about any of this. With my husband and my college friends I'm loud and funny. It's being judged by people at work or strangers that I can't handle.

Only if asked (don’t volunteer)

- Attacks happen only in social or performance situations
- Avoidance of speaking and eating in front of others since college
- Relaxed and confident with close friends and family

Patient 10: answer and debrief

DIAGNOSIS

Social anxiety disorder

Sudden surges of fear with normal medical tests suggest panic disorder. Clue 4 flips it: the attacks happen only when she might be watched or judged, and her fear is embarrassment. That is social anxiety disorder with panic attacks, not panic disorder, which requires unexpected attacks. She is at ease in close relationships and does not feel inferior overall, which argues against avoidant personality disorder. Exam lesson: ask what triggers the attacks and what the person fears.

What had to be ruled out

- Anxiety disorder due to another medical condition: normal heart and thyroid checks
- Stimulant intoxication or caffeine effects: one coffee a day, no other stimulants
- Panic disorder: attacks are cued by social scrutiny, not unexpected
- Agoraphobia: no fear of crowds, transit or being out alone
- Avoidant personality disorder: comfortable in close relationships; no pervasive sense of inadequacy

Checkpoint: What would you ask or check NEXT?

A. Teach her paced breathing to use when an attack starts

B. Ask exactly when and where the attacks happen and what she fears will happen **(best)**

C. Refer her to her doctor to discuss medication for the attacks

Panic attacks can occur in many disorders. Whether they are unexpected or tied to a feared situation decides the diagnosis, so assessment comes first. A and C are interventions chosen before the problem is understood.

The case in six clues (as the app reveals it)

1. My heart pounds, I sweat, my hands shake, and I'm sure I'm going to pass out. It comes on fast and peaks within a few minutes.
2. My doctor checked my heart and thyroid last month, and both were normal. I have one coffee in the morning and no other drugs or energy drinks.
3. It's happened maybe a dozen times since spring. I've started making excuses so I don't have to go certain places.
4. It never comes out of nowhere. It's in meetings when I have to present, or when the team lunch turns to me. I'm terrified they'll see me shake and think I'm a fraud.
5. It's not new. In college I dropped a class to skip a speech, and I've always eaten lunch at my desk. Now that I lead a team, it's every day.
6. With my husband and my old friends I'm relaxed and funny. I don't feel inferior as a person; it's being watched and judged by coworkers or new people that I can't stand.

What a skilled clinician weighs after each clue

1. Panic disorder, Anxiety disorder due to another medical condition, Stimulant intoxication
2. Panic disorder, Social anxiety disorder, Generalized anxiety disorder
3. Panic disorder, Agoraphobia, Social anxiety disorder
4. Social anxiety disorder, Panic disorder, Avoidant personality disorder
5. Social anxiety disorder, Avoidant personality disorder
6. Social anxiety disorder

Sources: DSM-5-TR (APA, 2022), Anxiety Disorders: Social Anxiety Disorder; DSM-5-TR (APA, 2022), Anxiety Disorders: Panic Disorder

Patient 11: The broken pencil

Setting: Isabela, 9, in fourth grade, at a children's outpatient clinic with her grandmother, who has raised her since she was two.

> *"Grandmother: I love this girl more than my life, but I don't know what to do anymore. Everyone in the family is afraid of her temper."*

If asked about…

When did this start?	Right after she turned eight, about a year and a half ago. There hasn't been one calm month since.
How often are the outbursts?	Three or four big ones a week at home, screaming, throwing things. The school says the same.
Mood between outbursts	She's cranky and snapping at everybody almost every day. She's not a happy kid anymore. Even on her birthday she was sour.
Periods of high mood or little sleep	Never. No silly-happy days, no nights awake full of energy, no talking a mile a minute. She's just angry.
Sleep	In bed at eight-thirty, up at seven. She sleeps like a rock.
Medical history and medications	Her pediatrician saw her in the summer. Hearing and vision are fine. No medicines, no head injuries, no seizures.
Safety	When she's mad she says 'I hate everyone.' She has never said she wants to die or hurt herself. She kicked her cousin once. Nobody's been badly hurt.
Family history	Her mother, my daughter, has bipolar disorder and lives in another state. Isabela's father isn't in the picture.
School	Her teacher says she's bright, but she's sent to the office twice a week. She can sit and draw for an hour. The other kids don't invite her to parties anymore.
Stressors or trauma	Nothing happened that I know of when it started. No abuse, no one new in the house, no move. She has lived with me since she was two.
Culture and context	We're Puerto Rican. In our house feelings come out fast, and then they're over. This is not that. My other grandkids get mad and it's over in five minutes. With her it never ends.

Only if asked (don’t volunteer)

- No distinct periods of elevated mood or reduced sleep
- Onset just after age 8; no symptom-free month since
- Outbursts at home, at school and with peers (classmates and cousins)

Patient 11: answer and debrief

DIAGNOSIS

Disruptive mood dysregulation disorder

A parent with bipolar disorder makes pediatric bipolar the early worry. Clue 4 flips it: there are no distinct episodes, only chronic irritability between frequent outbursts. Outbursts 3 or more times a week, irritable mood most days, onset before age 10, over 12 months with no calm stretch of 3 months, in more than one setting: DMDD. Exam lesson: DMDD was created to reduce overdiagnosis of bipolar in children, and when ODD criteria are also met, diagnose DMDD only.

What had to be ruled out

- Bipolar I disorder: no distinct episodes of elevated mood or reduced need for sleep
- Intermittent explosive disorder: mood stays irritable between outbursts
- Oppositional defiant disorder: DMDD takes precedence when both are met
- ADHD: can focus for long periods; teachers do not report hyperactivity
- Trauma-related or adjustment disorder: no identified trauma or stressor at onset

Checkpoint: What would you ask or check NEXT?

A. Refer her for a psychiatric evaluation for bipolar disorder given the family history

B. Start a home behavior chart with rewards for calm days

C. Ask whether she has had distinct periods of days with unusually high mood, energy and little sleep **(best)**

Family history raises concern, but bipolar disorder requires distinct episodes; chronic, non-episodic irritability points elsewhere. Asking about distinct episodes is the deciding assessment. A acts on family history before that key fact is known. B is an intervention chosen before the diagnosis is clear.

The case in six clues (as the app reveals it)

1. Grandmother: She explodes over little things. Last week she threw a chair because her pencil broke.
2. Grandmother: Her mother has bipolar disorder. I'm scared this is the same thing starting.
3. Teacher's note: At school it's three or four meltdowns a week, yelling and ripping up papers. Between them she's sour and snappy with everyone, almost every day.
4. Grandmother: No, she has never had days of being silly-happy or going without sleep. She sleeps fine. She's just angry, every day.
5. Grandmother: It started right after she turned eight, about a year and a half ago. There hasn't been one calm month since.
6. Teacher's note: Her outbursts are far beyond anything I see in other nine-year-olds. They happen with classmates at recess, not just with adults.

What a skilled clinician weighs after each clue

1. Oppositional defiant disorder, Intermittent explosive disorder, Disruptive mood dysregulation disorder, ADHD
2. Bipolar I disorder, Disruptive mood dysregulation disorder, Intermittent explosive disorder
3. Disruptive mood dysregulation disorder, Bipolar I disorder, Oppositional defiant disorder
4. Disruptive mood dysregulation disorder, Oppositional defiant disorder
5. Disruptive mood dysregulation disorder, Oppositional defiant disorder
6. Disruptive mood dysregulation disorder

Sources: DSM-5-TR (APA, 2022), Depressive Disorders: Disruptive Mood Dysregulation Disorder

Patient 12: The warehouse leg

Setting: Mateo, 44, a warehouse worker and father of three, referred to a hospital outpatient social worker by his neurologist. His wife is with him.

> *"Doctors keep saying my tests are normal. My leg is not normal. I don't know why I'm talking to a social worker."*

If asked about…

When did this start?	Six weeks ago, lifting boxes. No fall, no pop, no pain. It just went weak and heavy, and it's been the same since.
What it feels like	Like the leg belongs to someone else. I drag it. No numbness in my face, no trouble talking or seeing.
Falls	I've fallen twice at night going to the bathroom. My wife found me on the floor.
Tests	MRI, nerve tests, blood work, all normal. She had me push down with my good leg, and she said the weak leg pushed back hard without me trying. She said the wiring isn't damaged but the signal isn't getting through. I don't really get it.
The claim	HR filed workers' comp because it happened on the job. My cousin's friend is the lawyer. I'd rather be working. I go to every PT session and push as hard as I can.
Stress before it started	Honestly, nothing big. Money's tight like always. My kids are good. My marriage is good.
Worry about health	I don't look things up online. I'm not scared I'm dying. I'm frustrated. I want it fixed.
Sleep and mood	I sleep okay. I'm a little down because I can't work, but I still enjoy my kids and watching soccer.
Alcohol, drugs, medications	A couple of beers on Sundays. No drugs. Ibuprofen sometimes.
Medical history	Healthy. No seizures, no strokes. My father had a stroke at 70, so yes, this scared me at first.
Thoughts of suicide	No, never. I just want my leg back.
Culture and context	I came from El Salvador at 17 and I've worked every day since. In my family a man works. My mother thinks it's 'susto' from almost dropping that pallet, and she wants me to see a healer. Sitting home while my wife works double shifts is killing me.

Only if asked (don’t volunteer)

- The claim was filed by HR, not by him
- Falls at home when unobserved
- No health worry; no doctor visits for 10 years before this
- Weak leg pushes back strongly when he presses with the good leg (neurologist's exam)

Patient 12: answer and debrief

DIAGNOSIS

Functional neurological symptom disorder

Normal tests plus a finding that does not fit nerve disease point to a functional disorder. Clue 3 flips the lead to malingering: there is money at stake. Then the facts turn back: he falls when unobserved, works hard in therapy, did not file the claim, and wants to work. Without excessive health worry, somatic symptom disorder fits less well. Exam lesson: FND needs a positive exam finding that does not fit neurological disease. It is not faking, a psychological stressor is not required, and an incentive alone does not make someone a malingerer.

What had to be ruled out

- Neurological disease: normal MRI and nerve tests; findings inconsistent with nerve damage
- Malingering: symptoms persist unobserved, he did not seek the claim, and he wants to return to work
- Factitious disorder imposed on self: no evidence of faking or inducing symptoms
- Somatic symptom disorder: no excessive thoughts, worry or time devoted to health

Checkpoint: What would you ask or check NEXT?

A. Gather more information: how the weakness behaves across settings and how he understands it **(best)**

B. Tell him the tests show nothing is wrong with his leg

C. Report possible fraud to the workers' compensation insurer

A financial incentive raises the question of malingering but does not prove it, so collect more information before concluding anything. B is inaccurate and dismissive; the findings show a real functional problem, not 'nothing.' C acts on suspicion alone and breaches confidentiality without a basis.

The case in six clues (as the app reveals it)

1. My left leg gave out about six weeks ago while I was stacking boxes at work. Now I can barely walk without a cane.
2. Neurologist's note: Brain and spine MRI and nerve tests are normal. When he presses down with his good leg, the 'weak' leg pushes back strongly without his noticing.
3. Wife: He has a workers' comp claim open, and the lawyer says it could be worth a lot.
4. Physical therapist: He works hard in every session and gets frustrated when the leg won't cooperate. His wife has found him fallen on the bathroom floor at night when no one was watching.
5. HR filed the claim, not me. I'd give every dollar back to walk right and get back to work; my family needs my paycheck, not a settlement.
6. I'm not a worrier about my health. Before this I hadn't seen a doctor in ten years, and nothing big was going on in my life when it started.

What a skilled clinician weighs after each clue

1. Functional neurological symptom disorder, Malingering, Somatic symptom disorder
2. Functional neurological symptom disorder, Malingering, Factitious disorder imposed on self
3. Malingering, Functional neurological symptom disorder, Factitious disorder imposed on self
4. Functional neurological symptom disorder, Malingering, Somatic symptom disorder
5. Functional neurological symptom disorder, Somatic symptom disorder
6. Functional neurological symptom disorder

Sources: DSM-5-TR (APA, 2022), Somatic Symptom and Related Disorders: Functional Neurological Symptom Disorder (Conversion Disorder); DSM-5-TR (APA, 2022), Other Conditions That May Be a Focus of Clinical Attention: Malingering